

Suicide patterns among physicians related to other academics as well as to the general population. Results from a national long-term prospective study and a retrospective study.

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In the present study, we have followed a national cohort of physicians, academics and the general population (part of the compulsory census in 1960) for a period of 10 years and identified all cases of suicide during the period 1961-1970. Furthermore, we have carried out a retrospective study of suicides among the four major medical specialist groups (general practitioners, internists, psychiatrists and general surgeons) and compared these rates with other medical specialists. Results show an elevated standardized mortality ratio (SMR) for suicide among female physicians compared to other academics as well as to the general population. Furthermore, male doctors exhibit an elevated suicide rate only when compared to other academics. Among the various specialists, general surgeons alone exhibited a significantly elevated suicide rate. The study clearly shows that female physicians are more prone to suicide than most other women, but that male physicians are also at risk compared to other male academics. Furthermore, at least in Sweden, general surgeons, not psychiatrists, have the highest suicide rate of all physicians.

Suicide mortality among medical doctors in Finland: are females more prone to suicide than their male colleagues?

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Suicide mortality among medical practitioners is in many countries significantly higher compared with other professionals and the general population. Differences between male and female physicians are difficult to estimate reliably because previous comparisons are mainly based on crude mortality rates. Age-specific mortality rates were calculated for physicians, other professionals and the general population, males and females separately, as well as standardized mortality ratios (SMR) comparing physicians with the other groups. Crude mortality rates were calculated for the specialist groups. The SMR for male (female) physicians was 0.9 (2.4) compared with the general male (female) population and 2.4 (3.7) compared with other male (female) professionals. The SMR between male and female physicians was 1.2 (95% CI 0.9-1.7). Our results do not support the claim that female physicians have a greater risk of suicide than their male colleagues, but are concordant with previous observations of a higher suicide rate in female physicians compared with the general population and other female professionals.

Suicide rates among physicians: a quantitative and gender assessment (meta-analysis).

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Physicians' suicide rates have repeatedly been reported to be higher than those of the general population or other academics, but uncertainty remains. In this study, physicians' suicide rate ratios were estimated with a meta-analysis and systematic quality assessment of recent studies. Studies of physicians' suicide rates were located in MEDLINE, PsycINFO, AARP Ageline, and the EBM Reviews: Cochrane Database of Systematic Reviews with the terms "physicians," "doctors," "suicide," and "mortality." Studies were

included if they were published in or after 1960 and gave estimates of age-standardized suicide rates of physicians and their reference population or reported extractable data on physicians' suicide; 25 studies met the criteria. Reviewers extracted data and scored each study for quality. The studies were tested for heterogeneity and publication bias and were stratified by publication year, follow-up, and study quality. Effect sizes were pooled by using fixed-effects (women) and random-effects (men) models. The aggregate suicide rate ratio for male physicians, compared to the general population, was 1.41, with a 95% confidence interval (CI) of 1.21-1.65. For female physicians the ratio was 2.27 (95% CI=1.90-2.73). Visual inspection of funnel plots from tests of publication bias revealed randomness for men but some indication of bias for women, with a relative, nonsignificant lack of studies in the lower right quadrant. Studies on physicians' suicide collectively show modestly (men) to highly (women) elevated suicide rate ratios. Larger studies should help clarify whether female physicians' suicide rate is truly elevated or can be explained by publication bias.

Suicide compared to other causes of mortality in physicians.

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Physicians frequently are early adopters of healthy behaviors based on their knowledge and economic resources. The mortality patterns of physicians in the United States, particularly suicide, have not been rigorously described for over a decade. Previous studies have shown lower all-cause mortality among physicians yet reported conflicting results about cause-specific mortality such as suicide. In this study, we compared all-cause and cause-specific mortality in a sample of physicians to the age-gender matched general U.S. population from 1948 through 1998. We also compared the mortality experience across medical specialties. The risk of all-cause mortality was 56% lower than expected in men, and 26% lower in women, compared to the general population. Standardized mortality ratios (SMRs) were markedly lower for diseases strongly linked to smoking, e.g., cardiovascular diseases, respiratory diseases, and lung cancer. Suicide was the only cause of death where risk was greater than the general population. Overall, we found that physicians are at substantially lower risk of dying compared to the general population for all causes of death except suicide. The findings for suicide are strikingly different than other causes of death and should provide impetus for new research on the mental health of physicians.

Suicide, affective disorder, and women physicians.

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Recent publications have noted a rate of suicide for women physicians considerably higher than that for women in the general population. The authors comment on some of the methodologic problems involved in this research and discuss the interaction between affective disorder and suicide as but a partial explanation of the higher risk of suicide among women physicians. The authors conclude that a number of risk factors are involved both in the relatively high lethality rate of some groups of professional women and in the relatively low lethality rate of women in general.

Factors associated with suicide in physicians: a silent stigma and public health problem

that has not been studied in depth.

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Suicide is a complex and multifaceted public health issue that affects individuals from all walks of life, including healthcare professionals such as physicians. According to research, physicians have a higher risk of suicide compared to the general population, with an estimated suicide rate that is two to three times greater than that of the general population. Suicide in physicians can have devastating consequences, not only for the individual but also for their patients and colleagues. The factors contributing to suicide in physicians are numerous and often interrelated. Physicians are exposed to numerous stressors in their daily lives, including long work hours, high workload, burnout, and exposure to traumatic events. These stressors can lead to mental health problems such as depression, anxiety, and substance use disorders, which in turn can increase the risk of suicide. In addition to work-related stressors, personal factors such as relationship problems, financial stress, and a history of mental health problems can also contribute to suicide risk in physicians. Stigma and shame around seeking help for mental health issues may also prevent physicians from seeking treatment, exacerbating the problem. Understanding the complex factors that contribute to suicide in physicians is crucial for developing effective prevention strategies. For this reason, it is necessary to know the behavior of this phenomenon and the factors associated with a higher risk of suicide in this population. However, taking into account that different regions of the world vary in socioeconomic, cultural, professional, occupational, and health attributes, it is to be expected that the behavior of these risk factors will also be heterogeneous. At present, it is presumed that there is a significant gap in the evidence, due to a predominance of evidence on this topic from high-income countries. Considering the importance of having a comprehensive understanding of the risk factors for suicide in the medical population and possible strategies to mitigate this condition, the aim of this review is to analyze the most recent evidence on these factors, and to assess the quality of the evidence and gaps that need to be studied further.

Challenges and potential solutions for physician suicide risk factors in the COVID-19 era: psychiatric comorbidities, judicialization of medicine, and burnout.

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Suicide among physicians constitutes a public health problem that deserves more consideration. A recently performed meta-analysis and systematic review evaluated suicide mortality in physicians by gender and investigated several related risk factors. It showed that the post-1980 suicide mortality was 46% higher in female physicians than among women in the general population, while the risk in male physicians was 33% lower than among men in general, despite an overall contraction in physician mortality rates in both genders. This narrative review was conducted by searching and analyzing articles/databases that were relevant to addressing questions raised by a prior meta-analysis and how they might be affected by COVID-19. This process included unstructured searches on Pubmed for physician suicide, burnout, judicialization of medicine, healthcare organizations, and COVID-19, and Google searches for relevant databases and medical society, expert, and media commentaries on these topics. We focus on three factors critical to addressing physician suicides: epidemiological data limitations, psychiatric comorbidities, and professional overload. We found relevant articles on suicide reporting, physician mental health, the effects of healthcare judicialization, and organizational involvement on physician and patient health, and how COVID-19 may impact such factors. This review addresses information sources, underreporting/misreporting of physician suicide rates, inadequate diagnosis and management of psychiatric comorbidities and the chronic effects on physicians' work

capacity, and, finally, judicialization of medicine and organizational failures increasing physician burnout. We discuss these factors in general and in relation to the COVID-19 pandemic. We present an overview of the above factors, discuss possible solutions, and specifically address how COVID-19 may impact such factors.

The prevalence of suicidal ideation and suicidal attempts among Norwegian physicians. Results from a cross-sectional survey of a nationwide sample.

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Physicians have a higher suicide rate than the general population or other academics. Little is known about the reasons for this. Analysing risk factors may be a valuable way of identifying reasons for the high suicide rate among physicians, thereby leading to preventive efforts. The present study is one of the first papers on suicidal thoughts and attempts among physicians. A questionnaire about suicidal thoughts (developed by E.S. Paykel) was completed by 1,063 of 1,476 active Norwegian physicians (72%). Lifetime prevalence ranged from 51.1% for feelings that life was not worth living to 1.6% for a suicide attempt. Risk factors were being female, living alone, and depression. Suicidal thoughts, however, were hardly attributed to working conditions. A high rate of suicide and a low rate of suicidal attempts support the hypothesis that physicians do not 'cry for help,' but are inclined to act out their suicidal impulses.

Physician suicide in North Carolina.

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Previous research suggests that physicians are two to three times as prone to suicide than the general population. Review of North Carolina death certificates from 1978 to 1982 showed that the physician suicide rate is not substantially different from that of the white male population aged 25 years or older. Old age, divorce, and death of a spouse are associated with the highest suicide rates among physicians. Firearms and drugs are the most common suicidal methods. Depression, substance abuse, and retirement are significantly associated with suicide risk. Physicians need to recognize the existence of increased suicide risk with age and retirement.

[Not enough help for themselves--the risk of physicians to commit suicide].

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Physicians seem unwilling to deal with their own suicidal problems professionally. Suicide is a repressed topic. According to international studies, medical students and physicians are clearly over-represented among suicide victims. Committing suicide stands in sharp contrast to the positive image physicians enjoy as competent, strong helpers transmitting positive energy. Various studies and meta-analyses show that physicians use knowledge specific to their profession and are therefore "more successful" than the general population in committing suicide. Moreover, the data reveal a number of risk factors specifically correlating with medical practice. This is confirmed by an increased number of suicides during medical training and professional life in comparison with the general population. Gender-specific analyses

show an even higher suicide risk for female physicians. In this context it cannot be excluded that out of "professional respect" cardiovascular causes of death are sometimes falsely documented in death certificates instead of suicide. Despite their special education, physicians are not very good at diagnosing their own emotional disorders and asking colleagues for adequate professional help. They rather tend to camouflage their own psychological problems also because they are afraid of occupational and personal discrimination.
